

PATIENT REFERRAL DOCUMENT

LMU Klinikum Munich

Patient: Herbert Zimmermann

Date of Birth: 03.04.1958

Date: 15.05.2026

Referring Physician: Dr. Maria Hofmann

General Practice Bogenhausen, Munich

REASON FOR REFERRAL:

Patient with known chronic kidney disease stage 3b, hypertension, and type 2 diabetes mellitus requiring specialist nephrology review. eGFR has declined from 42 to 36 mL/min over the past 18 months. Patient remains asymptomatic and clinically stable. Referral for specialist input on optimising management to slow disease progression and plan for future renal replacement therapy if required.

CLINICAL FINDINGS:

Blood pressure: 138/84 mmHg · on three antihypertensive agents

Heart rate: 72 bpm

Temperature: 36.8 degrees Celsius

Oxygen saturation: 98% on room air

Weight: 84kg, Height: 178cm, BMI: 26.5

No peripheral oedema

No signs of fluid overload

RENAL EXAMINATION:

Abdomen soft, non-tender

No palpable kidneys

No bruits heard over renal arteries

Urine dipstick: Protein 2+, Blood trace

CURRENT MEDICATIONS:

Ramipril 10mg daily · hypertension and renoprotection

Amlodipine 10mg daily · hypertension

Furosemide 40mg daily · fluid management

Metformin 500mg twice daily · type 2 diabetes

Atorvastatin 20mg daily · cardiovascular risk

Calcium carbonate 500mg three times daily · phosphate binder

Alfacalcidol 0.25 mcg daily · vitamin D supplementation

PAST MEDICAL HISTORY:

Chronic kidney disease stage 3b · diagnosed 2019

Type 2 diabetes mellitus · diagnosed 2012, on oral therapy

Hypertension · diagnosed 2010

Anaemia of chronic disease · managed with dietary advice

No previous episodes of acute kidney injury

No urological procedures

INVESTIGATIONS · MOST RECENT (April 2026):

Creatinine: 178 umol/L · up from 158 umol/L 18 months ago

eGFR: 36 mL/min · down from 42 mL/min 18 months ago

Urea: 12.4 mmol/L

Potassium: 4.8 mmol/L · upper limit of normal

Sodium: 139 mmol/L
Bicarbonate: 22 mmol/L · mild metabolic acidosis
Haemoglobin: 10.2 g/dL · anaemia present
Ferritin: 68 ug/L
PTH: 8.2 pmol/L · mildly elevated
Phosphate: 1.52 mmol/L · upper limit of normal
HbA1c: 7.4% · reasonable diabetic control
Urine albumin creatinine ratio: 68 mg/mmol · significantly elevated

FAMILY HISTORY:

Brother · end stage renal disease on dialysis
Mother · hypertension, deceased aged 79

SOCIAL HISTORY:

Retired civil servant
Non-smoker
Alcohol · 8 units per week
Lives with wife, two adult children

CLINICAL IMPRESSION:

Patient with progressive CKD stage 3b with declining trajectory. Worsening proteinuria and rising PTH suggest need for specialist input on phosphate management, anaemia of chronic kidney disease, and planning for future renal replacement therapy options. Potassium trending upward · may need medication review. Patient would benefit from nephrology clinic involvement for ongoing monitoring and patient education regarding dialysis options.

Urgency: Routine · please see within 2 weeks.